

INTERNSHIP REPORT

ADR REPORTING AND DOCUMENTATION

A Comprehensive Internship Task Report

Submitted By	_____
Course / Program	_____
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Internship Duration	_____
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1. Introduction

An Adverse Drug Reaction (ADR) is a harmful and unintended response to a medicinal product occurring at doses normally used in humans for prevention, diagnosis, treatment of disease, or modification of physiological function. ADR reporting is an important component of pharmacovigilance because information from individual cases can help identify previously unrecognized safety problems, changes in the frequency or severity of known reactions, and populations that may be at increased risk.

A structured reporting and documentation system ensures that suspected ADRs are identified promptly, communicated to the appropriate pharmacovigilance unit, evaluated, and followed up when necessary. Good documentation also supports patient safety, clinical decision-making, regulatory action, and learning within the healthcare organization.

2. Objectives

- To understand the concept and importance of ADR reporting.
- To learn how suspected ADRs are identified and reported.
- To understand the essential information required for a valid ADR report.
- To study documentation, assessment, follow-up, and record maintenance.
- To understand the role of pharmacists and other healthcare professionals in pharmacovigilance.
- To promote safe medication use and prevention of avoidable harm.

3. Importance of ADR Reporting

ADR reporting contributes to the continuous monitoring of medicine safety after a product is introduced into routine clinical use. Clinical trials may not detect rare, delayed, long-term, or population-specific adverse reactions because of limited sample size and follow-up duration.

Individual case safety reports, when collected and assessed systematically, can generate safety signals. These signals may lead to further investigation, updated prescribing information, additional monitoring, risk-minimization measures, or other regulatory and clinical actions.

Reporting is therefore useful even when the reporter is not certain that the medicine caused the event. The role of the reporter is generally to provide a reasonable clinical suspicion with accurate information rather than to prove causality.

4. Common Types / Categories of ADRs

- Type A (Augmented): Dose-related and generally predictable from the known pharmacology of the medicine.
- Type B (Bizarre): Unpredictable reactions, often unrelated to the usual pharmacological action; examples include some allergic or idiosyncratic reactions.
- Type C (Chronic): Associated with long-term use and cumulative exposure.
- Type D (Delayed): Become apparent after a delay, sometimes after exposure has ended.

- Type E (End-of-use): Associated with withdrawal or discontinuation of a medicine.
- Type F (Failure): Unexpected failure of therapy, sometimes related to interactions, resistance, or other factors.

5. ADR Reporting Process

1. Detection: Identify a suspected adverse event through patient history, clinical observation, laboratory results, medication review, or patient/caregiver communication.
2. Initial assessment: Confirm the event, review the medication history, and consider alternative causes and relevant clinical factors.
3. Documentation: Record the patient's details, suspected medicine(s), reaction, timing, management, outcome, and relevant investigations.
4. Reporting: Submit the suspected ADR through the appropriate institutional, national, or regulatory pharmacovigilance reporting mechanism.
5. Follow-up: Obtain missing or updated information where appropriate, particularly for serious, medically important, or incomplete cases.
6. Assessment and signal detection: The pharmacovigilance team assesses the case and aggregates reports to identify safety signals and trends.
7. Action and feedback: Where required, appropriate clinical, institutional, or regulatory risk-minimization actions are taken and learning is communicated.

6. Essential Information in an ADR Report

A useful ADR report should contain sufficient information to allow the case to be understood and assessed. The core elements commonly include an identifiable patient, an identifiable reporter, a suspected medicinal product, and a description of the suspected adverse reaction.

Information Area	Examples of Information
Patient	Age, sex, relevant medical history, weight where relevant, and other clinically useful identifiers (avoid unnecessary personal identifiers).
Suspected medicine	Generic/brand name, strength, dosage form, dose, route, frequency, indication, start/stop dates, batch/lot number when relevant.
Reaction	Description of signs/symptoms, diagnosis if available, onset date, severity, seriousness, relevant laboratory findings.
Management	Treatment given, medicine withdrawn/reduced/rechallenged if applicable, hospitalization or other interventions.
Outcome	Recovered, recovering, not recovered, recovered with sequelae, fatal, or unknown.
Concomitant medicines	Other medicines, vaccines, supplements, or herbal products used around the event.
Reporter	Name/role, contact details, department/institution, and date of report, as required by the reporting system.

7. Seriousness vs Severity

Severity describes the intensity of a reaction (for example, mild, moderate, or severe). Seriousness is a regulatory/clinical concept based on outcomes or criteria such as death, life-threatening condition, hospitalization or prolongation of hospitalization, persistent/significant disability, congenital anomaly, or another medically important event. These terms should not be used interchangeably.

8. Documentation Standards

ADR documentation should be clear, factual, chronological, legible, and sufficiently detailed. Record what was observed or reported rather than making unsupported assumptions. Dates and times should be included when they materially help establish the exposure–event relationship.

Corrections in paper records should follow institutional documentation policy. Electronic records should preserve an audit trail where applicable. Confidentiality must be maintained, and patient information should only be shared through authorized channels.

Relevant source documents may include medication charts, prescriptions, laboratory reports, progress notes, discharge summaries, nursing observations, and patient statements. The ADR report should be consistent with the source record.

9. Assessment of a Suspected ADR

Assessment may consider the temporal relationship between medicine exposure and reaction, known pharmacological effects, alternative explanations, dechallenge/rechallenge information where available, dose-response relationship, laboratory or diagnostic evidence, and the patient's underlying disease.

Causality assessment can be performed using a structured method adopted by the institution or pharmacovigilance program. The assessment should remain transparent about uncertainty; a suspected report can still be valuable even when causality is not established.

10. Role of the Pharmacist / Healthcare Professional

- Monitor patients for suspected medicine-related problems.
- Review medication history and identify potential drug-related causes.
- Encourage timely reporting by healthcare staff and patients where applicable.
- Ensure reports contain accurate and clinically relevant information.
- Maintain confidentiality and appropriate records.
- Participate in follow-up and case assessment.
- Educate patients and healthcare teams about medicine safety.
- Help identify preventable ADRs, medication errors, interactions, and opportunities for safer therapy.

11. ADR Reporting in India

In India, pharmacovigilance activities are coordinated through the national pharmacovigilance framework, with the Pharmacovigilance Programme of India (PvPI) serving as a major national mechanism for ADR monitoring. Suspected ADRs can be reported by

healthcare professionals and consumers through the channels provided by the national program and participating centres.

During an internship, the student should follow the reporting pathway used by the hospital or institution and the current official PvPI/IPC requirements. Forms, contact details, submission routes, and procedures can change, so the latest official guidance should be checked before actual submission.

12. Sample ADR Case Documentation

Case: A 45-year-old patient develops a generalized itchy rash two days after starting a prescribed oral antibiotic. The patient has no documented previous allergy to the medicine. The medicine is stopped after clinical review, symptomatic treatment is provided, and the rash improves over the next few days.

Documentation points: Record patient age/sex; suspected antibiotic, strength, dose, route and indication; treatment start date; exact onset and description of rash; concomitant medicines; relevant history; management; dechallenge information; clinical outcome; reporter details; and any follow-up information.

Important: The case should be recorded as a suspected ADR. The report should not claim definite causality solely because the reaction followed medicine use.

13. Sample ADR Reporting Checklist

☐	Identifiable patient information recorded appropriately
☐	Reporter identity and contact information recorded
☐	Suspected medicine clearly identified
☐	Dose, route, frequency and indication documented
☐	Start/stop dates or duration of exposure recorded where available
☐	ADR description and onset documented
☐	Seriousness and clinical severity assessed appropriately
☐	Relevant investigations/laboratory results attached or referenced
☐	Concomitant medicines and relevant medical history recorded
☐	Management and dechallenge/rechallenge information recorded if applicable
☐	Outcome and follow-up requirements documented
☐	Confidentiality and institutional reporting procedure followed

14. Benefits of Proper ADR Documentation

Accurate documentation improves continuity of care, supports future prescribing decisions, provides evidence for pharmacovigilance assessment, helps prevent recurrence, and contributes to the wider knowledge base on medicine safety. Consistent records also make it easier to audit the quality of reporting and identify gaps in clinical practice.

15. Limitations and Common Reporting Problems

Under-reporting, incomplete information, poor chronology, missing dose/exposure details, failure to document concomitant medicines, and confusion between seriousness and severity can reduce the usefulness of reports. Lack of follow-up information may also make assessment difficult.

These limitations can be reduced through staff education, simple reporting pathways, prompt documentation, regular review of ADR registers, feedback to reporters, and a supportive safety culture.

16. Conclusion

ADR reporting and documentation are essential parts of pharmacovigilance and patient safety. A well-documented suspected ADR report provides a structured account of the patient, suspected medicine, reaction, clinical course, management, and outcome. Timely reporting, careful documentation, confidentiality, and appropriate follow-up help healthcare organizations detect safety concerns and reduce preventable medicine-related harm.

Interns should develop the habit of recognizing suspected ADRs, documenting them objectively, and following their institution's current reporting procedure. The goal is not merely to complete a form, but to create reliable safety information that can support better patient care.

17. References

1. World Health Organization (WHO). Pharmacovigilance: ensuring the safe use of medicines. WHO guidance and pharmacovigilance resources.
2. Indian Pharmacopoeia Commission (IPC). Pharmacovigilance Programme of India (PvPI): ADR monitoring and reporting resources.
3. Indian Pharmacopoeia Commission. Suspected Adverse Drug Reaction Reporting Form and related pharmacovigilance guidance.
4. Uppsala Monitoring Centre (UMC). Pharmacovigilance resources and individual case safety report concepts.
5. Relevant institutional Standard Operating Procedures (SOPs) for ADR reporting and medication safety.

Declaration: This report is prepared for academic/internship learning. Actual ADR reporting should follow the current approved procedure and official reporting forms of the relevant healthcare institution and pharmacovigilance authority.

Student Signature	Supervisor Signature
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Date: _____	Date: _____